

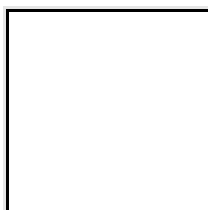
Debra Tucker

From: Heather Baisch
Sent: Thursday, February 09, 2017 2:09 PM
To: Debra Tucker; Kal, Khadeja
Subject: FW: News and policy updates - read our February newsletter and bulletin

FYI-please see IMRT info below. I could not get the link to the update to work.

From: Provider Communications [mailto:provider_communications@regence.com]
Sent: Wednesday, February 01, 2017 6:03 AM
To: Heather Baisch <heatherb@radiantcare.com>
Subject: News and policy updates - read our February newsletter and bulletin

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February 1, 2017

The February issues of *The Connection*SM and *The Bulletin* are now available. *The Connection* contains important information and notices of changes that impact your office. *The Bulletin* includes recent and upcoming changes to our medical and dental policies, and associated changes to pre-authorization requirements. Please be sure to read both publications.

The Connection articles include:

Feature

- Provider data accuracy is important

News and updates

- HEDIS reviews begin soon
- Provider Contact Center phone prompts

Pre-authorization

- Pre-authorization list updates

Pharmacy

- Medication policy update

- Newsletter and bulletin survey results

Administrative and billing updates

Policies

- Medical policy updates
- Clinical Practice Guidelines review
- Reimbursement policy updates
- Coding Toolkit updates
- Non-reimbursable services
- Diabetes management outcomes
- Changes to admission requirements
- Genetic testing for UMP members

- Administrative Manual updates
- Are your claims being returned?
- Washington providers invited to webinars

Products and Programs

- FEP disease management changes
- Medicare compliance reminder
- 2017 Medicare Quality Incentive Program
- MOON required for Medicare members
- New health trust joins Regence in April

Read the February issue of *The Connection*

The Bulletin

A supplement to *The Connection*, this monthly bulletin includes recent and upcoming changes to our medical and dental policies, and associated changes to pre-authorization requirements.

Note: For the Federal Employee Program® (FEP®), please refer to the [Blue Cross Blue Shield Service Benefit Plan brochure](#), the [FEP medical policies](#), or call our [customer service team](#).

Commercial medical policies

Changes effective January 1, 2017

Laboratory

- **Urine Drug Testing for Substance Abuse and Chronic Pain (#68)**
 - Replaced three presumptive testing codes with comparable codes
 - Added CPT® 80305-80307 and HCPCS G0659 with 15 unit per year limit

Changes effective January 6, 2017

Genetic Testing

- **Assays of Genetic Expression in Tumor Tissue as a Technique to Determine Prognosis In Patients With Breast Cancer (#42)**
 - Added EndoPredict® and Breast Cancer IndexSM tests to medical necessity criteria
 - Added language to clarify testing for patients with ductal carcinoma in situ (DCIS)

Changes effective February 1, 2017

Genetic Testing

- **Evaluating the Utility of Genetic Panels (#64)**
 - Added investigational panels
 - Updated the name of one panel

Laboratory

- **Analysis of Proteomic and Metabolomic Patterns for Early Detection of Cancer (#41)**
 - Added that metabolomic tests are considered investigational

Medicine

- **Intensity-Modulated Radiotherapy (IMRT) of the Abdomen and Pelvis (#139)**
 - Clarified criteria language regarding the need for submitted histogram analysis
- **Intensity Modulated Radiotherapy (IMRT): Central Nervous System (CNS) and Vertebral Tumors (#147)**
 - Added criteria that IMRT is medically necessary when there is documented prior radiation treatment to the planned target volume
 - Clarified language about the need for submitted histogram analysis
- **Intensity Modulated Radiotherapy (IMRT) of the Head and Neck Cancers and Thyroid Cancer (#138)**
 - Added criteria that IMRT is medically necessary when there is documented prior radiation treatment to the planned target volume
 - Clarified language regarding the need for submitted histogram analysis

Surgery

- **Bariatric Surgery (#58)**
 - Clarified that hiatal hernia repair includes either repair of sliding or paraesophageal hernia
- **Transesophageal Endoscopic Therapies for Gastroesophageal Reflux Disease (GERD) (#110)**
 - Liberalized investigational denial on CPT 43192 and 43201 to require pre-authorization

Changes effective May 1, 2017

Genetic Testing

- **Genetic Testing for Hereditary Hearing Loss (#36)**
 - Creating criteria for syndromic and nonsyndromic hearing loss (NSHL)
 - Clarifying use of single gene testing and multigene panel testing

Surgery

- **Transesophageal Endoscopic Therapies for Gastroesophageal Reflux Disease (GERD) (#110)**
 - Adding CPT 43236 with pre-authorization required

[View all commercial medical policy updates](#)

Medicare Advantage medical policies

Changes effective February 1, 2017

Genetic Testing

- **Genetic and Molecular Diagnostics – Next Generation Sequencing and Genetic Panel Testing** (Medicare #64)
 - Replaced the use of Noridian local coverage determination (LCD) L35008 with local coverage article (LCA) A55424 for the DecisionDx®-UM test for uveal melanoma

Surgery

- **Bariatric Surgery** (Medicare #58)
 - Clarified intent
 - Added reference for the treatment of complications related to prior bariatric procedures
 - Added requests for repeat bariatric procedures

Changes effective May 1, 2017

Surgery

- **Transesophageal Endoscopic Therapies for Gastroesophageal Reflux Disease (GERD)** (Medicare #110)
 - Adding pre-authorization requirement to CPT 43236

[View all Medicare Advantage medical policy updates](#)

Join our medical policy discussion

We welcome your input and feedback as we draft our medical policies. [Join our email reviewer list](#). While we prefer to receive input as policies are developed, we also have a formal process that allows you to submit additional information, such as clinical trial results, that may warrant a policy review.

Verify your provider information

Providing up-to-date and accurate information about the providers in each of our networks is a critical component of satisfaction for our members and a requirement for compliance with the Affordable Care Act (ACA) and Medicare Advantage plans.

Validating provider directory content

Please [follow these steps](#) to review the information about your practice every 30 days. If your information is correct, you do not need to do anything else at this time. We will verify the information you provide on a regular basis. Please respond timely to any requests from us for verification of your directory data.

Submitting changes or corrections

Please notify us if any information about your practice is incorrect or has changed by completing the [Provider information update form](#) for any of the following changes:

- Specialty
- Phone number
- Organization's address
- Accepting new patients
- Tax identification number
- eContracting email address
- National Provider Identifier (NPI) number
- Providers joining or leaving your clinic or practice

Note: If you are letting us know about a change in where your payments are directed, are closing a practice, terminating a network affiliation or are an organization that has changed ownership, please [contact your provider relations representative](#) instead of submitting the form.

If your clinic or facility submits provider rosters to us, please submit changes, corrections, additions or terminations immediately so that we can update the information that is displayed in our online directories as soon as possible. Your roster must be validated and reviewed in its entirety at least once per year.

We appreciate your assistance in keeping information about your practice up to date.

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